

Streamlining Hepatitis C Treatment: Implementation of a Specialty Pharmacist-Driven Clinical Program

Alexandra Ritenour, PharmD, CSP | Kiersi Harmon, PharmD, CSP
CHRISTUS Specialty Pharmacy – Tyler, Texas

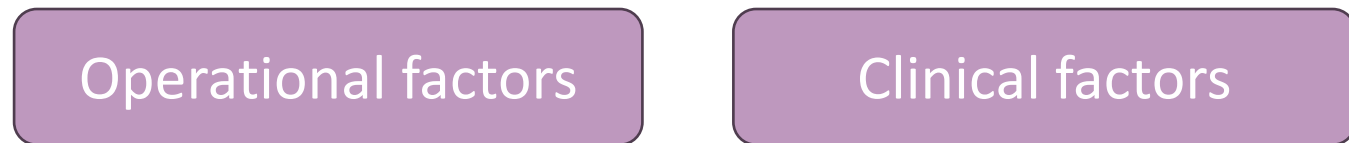


Background

Chronic Hepatitis C (HCV) Diagnosis & Treatment

- HCV impacts over 3.9 million people in the United States¹
- Of the 50% of patients diagnosed and aware of their HCV infection, only 17% of patients have been prescribed HCV treatment²
- Direct-acting antiviral (DAA) agents are highly effective in treating chronic HCV, achieving a treatment response rate above 90%, even in patients with previous treatment failures³

Pre-treatment Evaluation



- Clinical review of HCV treatment appropriateness, therapy selection, drug-drug interactions (DDIs) and navigation of medication access is a time-consuming process⁴

Specialty Pharmacist (SP) Place in Care & Gap in Knowledge

- SP may be the ideal provider to bridge gaps in HCV care through reducing time to therapy, selecting therapy, and insurance formulary and cost assistance navigation
- SP are often integrated within a specialty clinic as a member of the healthcare provider team
- Clinical benefits of SP in the HCV therapy selection process has been shown; however, impact on operational benefits and specifics on a SP workflow processes is limited^{2,5}

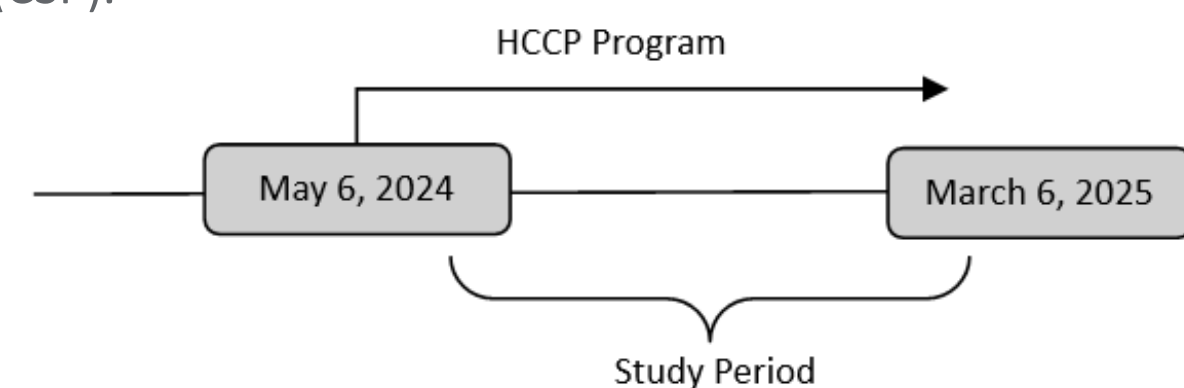
Objectives

This study aims to quantify and categorize SP-driven enrollments in a Hepatitis C Clinical Program (HCCP) and assess its impact on patient safety, operational, and clinical outcomes.

Methods

Study Design

This retrospective descriptive study aimed to summarize patient characteristics and clinical, operational, and safety outcomes among patients enrolled in the Hepatitis C Clinical Program (HCCP) within CHRISTUS Gastroenterology Clinics or filling HCV prescriptions at CHRISTUS Specialty Pharmacy (CSP).



Inclusion Criteria:

- Adults with diagnosis of chronic HCV (ICD-10 code, B18.2)
- Dual or triple all-oral DAA treatment prescribed from a CHRISTUS Gastroenterology clinic or filled at the CSP

Exclusion Criteria: Did not initiate HCV therapy following referral

Results

Hepatitis C Clinical Program

Figure 1

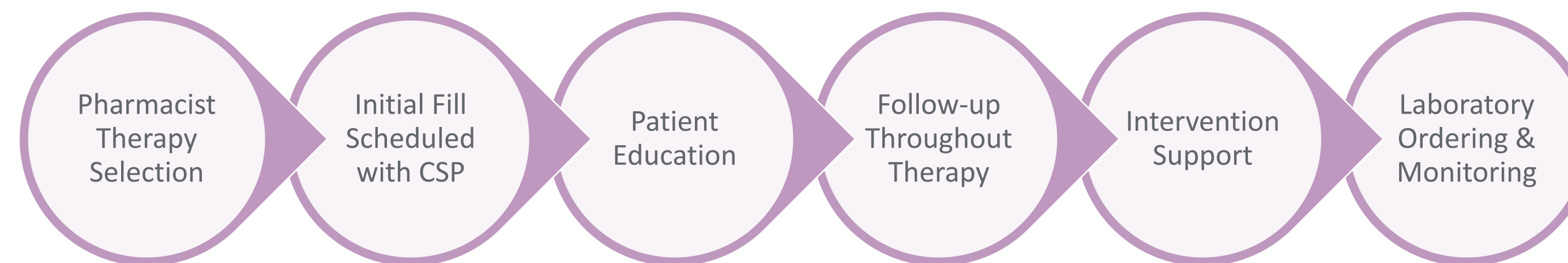
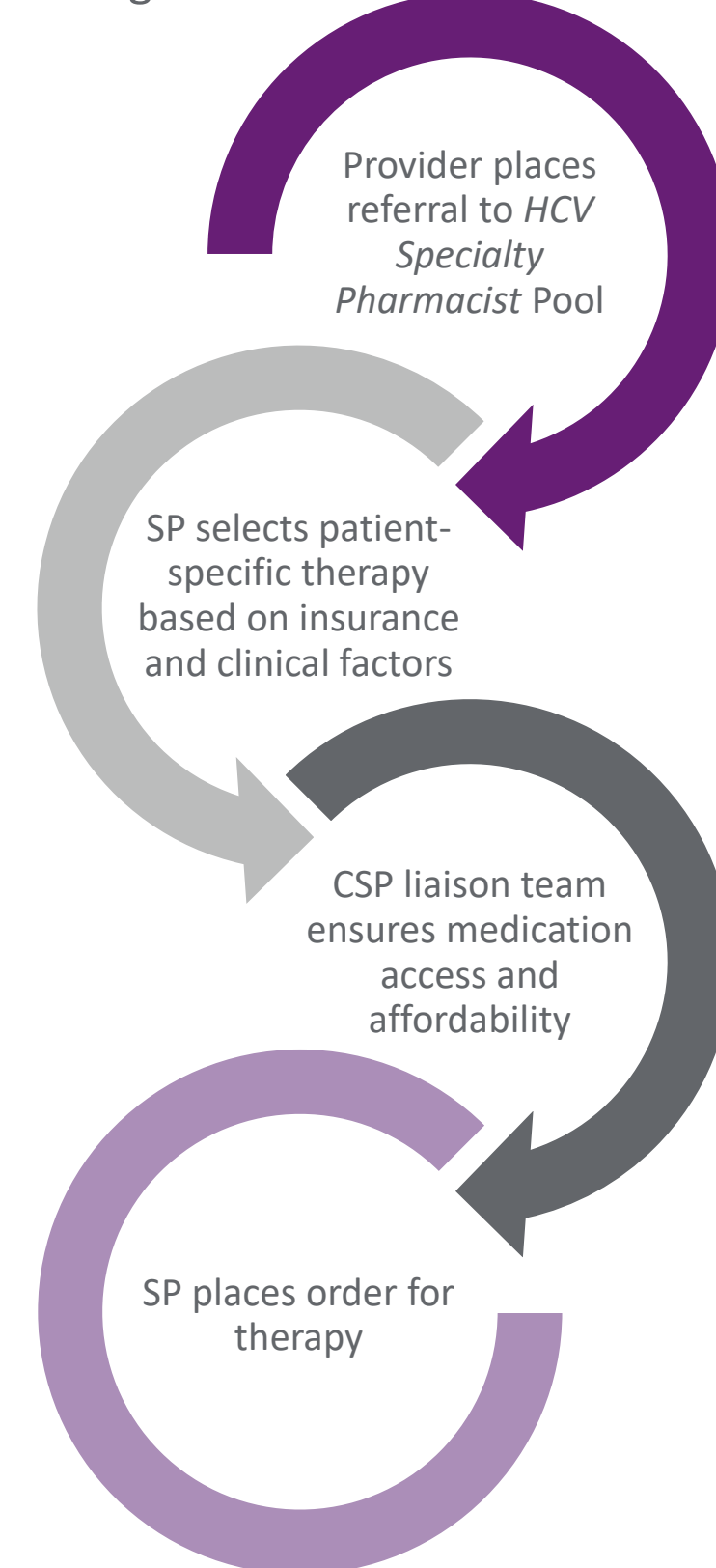


Figure 2

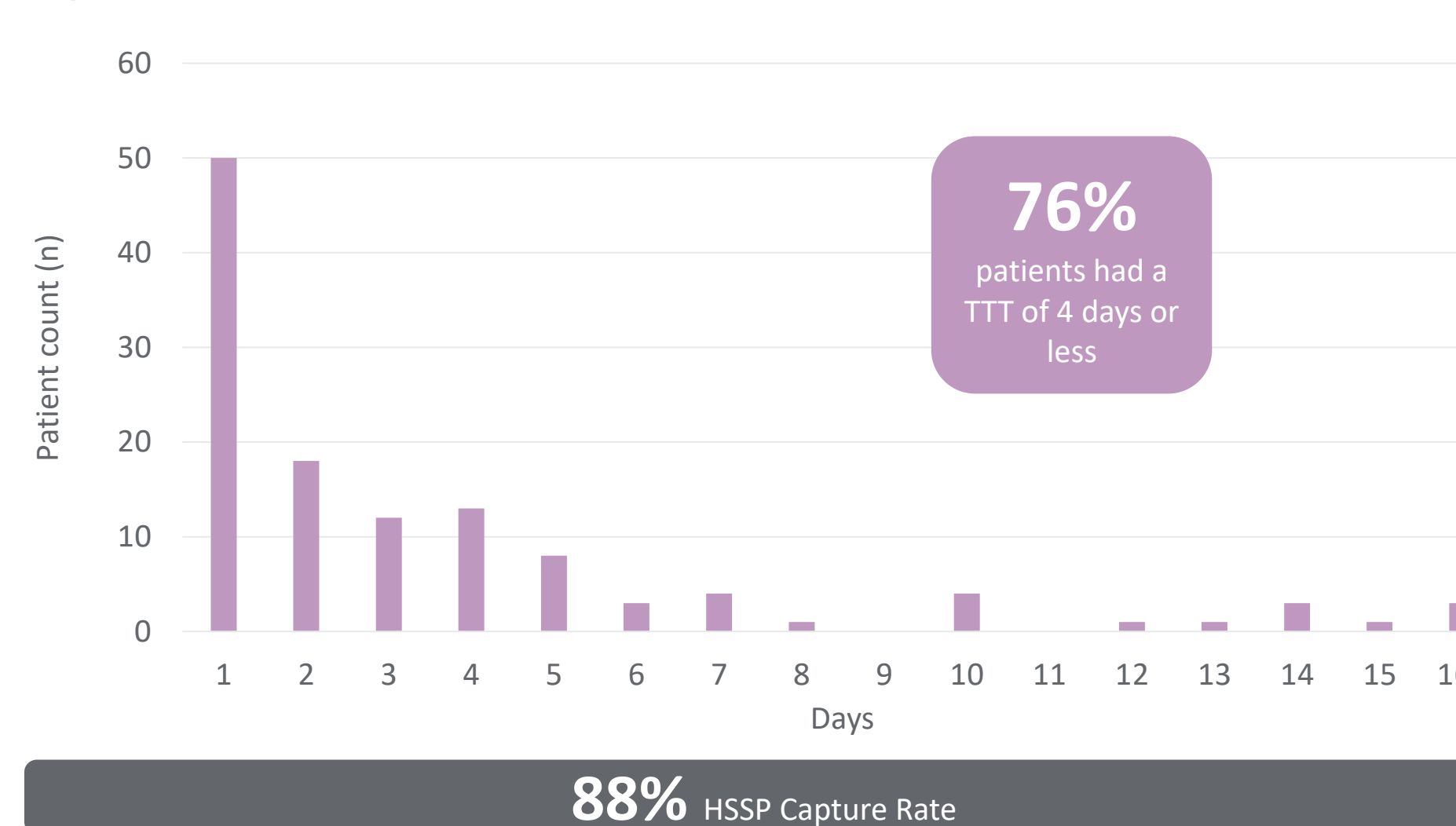


Patient Characteristics at Time of Enrollment

122 patients enrolled	90% HCV Treatment Naive
79% Non-Cirrhotic	66% Medicare
Table 1	
HCV Therapy	N (%)
SOF/VEL	65 (53)
G/P	45 (37)
SOF/VEL/VOX	9 (7)
SOF/VEL + RBV	2 (2)
G/P + RBV	1 (1)

Operational Outcomes

Figure 4. Time-to-Treatment (TTT)



Conclusions

- This study highlights the role of the SP in HCV care and their large impact on patient safety, clinical and operational outcomes.
- Results support further development of SP-driven practice changes and support the implementation of a similar program within other HSSPs.

Disclosures

The authors of this presentation have nothing to disclose concerning possible financial or personal relationships with commercial entities that may have a direct or indirect interest in the subject matter of this presentation.

Clinical Outcomes

57%

Required DDI Resolution by SP

Table 2

DDI	N
Statins	39
Proton Pump Inhibitors	28
Antipsychotics	2
Cardiac-related	6
Other	8

51%

DDIs Required External Provider Involvement for Resolution

92%

On-Time Therapy Completion

31%

Required SP Interventions During Therapy

97%

Achieved SVR12

Figure 3. Intervention Type

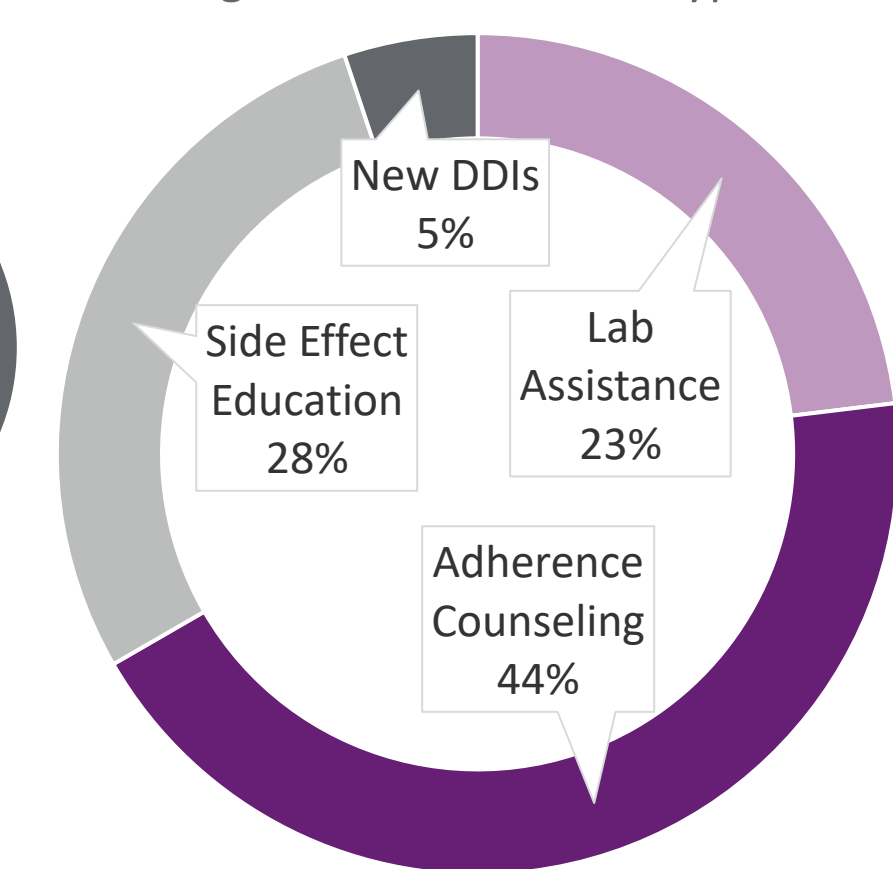


Figure 1. Overall HCCP Workflow **Figure 2.** Therapy selection process built in electronic health record (EPIC). Once SP places order for therapy, provider and nursing team are automatically alerted of order. **Table 1.** HCV Therapy distribution; SOF: sofosbuvir, VEL: velpatasvir, G/P: glecaprevir/pibrentasvir, VOX: voxilaprevir, RBV: ribavirin. **Table 2.** Distribution of DDIs with HCV therapy. **Figure 3.** SP Interventions by type. **Figure 4.** TTT defined as the difference in time between the referral placed to the HCV Specialty Pharmacist pool to time of SP placing therapy order.

References

- Hall E, Bradley H, Barker LK, et al. Estimating hepatitis C prevalence in the United States. *Hepatology* 2025;81(2):625-36.
- Koren D, Zuckerman A, Teply R, et al. Expanding hepatitis C virus care and cure: national experience using a clinical pharmacist-driven model. *Open Forum of Infectious Diseases* 2019;6(7):316.
- Dietz C, Maasoumy B. Direct-acting antiviral agents for Hepatitis C virus infection – from drug discovery to successful implementation in clinical practice. *Viruses* 2022;14(6):1325.
- Lo Re V 3rd, Gowda C, Urlick PN, et al. Disparities in absolute denial of modern hepatitis C therapy by type of insurance. *Clin Gastroenterol Hepatol* 2016; 14:1035-43.
- Zhu J, Hazen R, Joyce C, et al. Local specialty pharmacy and specialty clinic collaboration assists access to hepatitis c direct-acting antivirals. *Journal of the American Pharmacists Association* 2018;58:89-93.